M.PHIL. CLINICAL PSYCHOLOGY · STUDY NOTES

Psychotherapy — Unit I: Introduction to Psychotherapy

Definitions · Objectives · Training of therapists · Ethical & legal issues · Rights & responsibilities · Consent & assent · Planning & recording · Structure & goals · Pre/post-assessment · Evidence-based practice

Syllabus (Unit I): Introduction to Psychotherapy: Definitions, objectives, issues related to training professional therapists; ethical and legal issues involved in therapy work; rights and responsibilities in psychotherapy; issues related to consent (assent in case of minors); planning and recording of therapy sessions; structuring and setting goals; pre- and post-assessment; practice of evidence-based therapies.

Built from the 34+ psychotherapy books and the uploaded *notes_of_psychotherapies[1].pdf* in the repository, with primary documents of the RCI, IACP (as far as accessible), APA, BPS, WHO and Indian statutes. Page numbers are PDF page indices. See “How to read these notes” and the Source Audit.

Dango_Psych by Tejas Dangodra | M.Phil. Clinical Psychology

How to read these notes

These are concise, bullet-style revision notes for Unit I of the Psychotherapy paper (M.Phil. Clinical Psychology). They cover all ten syllabus topics and fold in the relevant regulatory and professional bodies (RCI, IACP, WHO and APA) on the topics where each is examinable.

Evidence labels used throughout

Each claim carries a label so that opinion is never presented as law.

Label	Meaning
BINDING LAW	A statute or court order that binds by force of law.
PROFESSIONAL CODE	An enforceable professional or ethics code of a regulator or association.
GUIDELINE	Aspirational or best-practice guidance; recommended, not mandatory.
THEORETICAL VIEW	A theoretical or author's position; schools differ.
ILLUSTRATIVE EXAMPLE	An invented teaching example; names and details are fictional.
SECONDARY SOURCE	Information available only through a secondary summary, not the primary document.

Citation convention

- Citations look like [Corey, PDF p.74-77]. “PDF p.” is the page index inside the source file in the repository, which usually differs from the printed page number (printed pages change between editions and are missing from reflowed e-books; the PDF index is exactly reproducible).
- Everything quotable is paraphrased; only verified page indices are cited. Where a page index was not verified, no page is given.

1 Definitions of psychotherapy

Psychotherapy = a planned, systematic, psychological (non-medical) treatment in which a trained professional uses a confiding relationship and theory-based methods to help a help-seeking client change feelings, thoughts and behaviour.

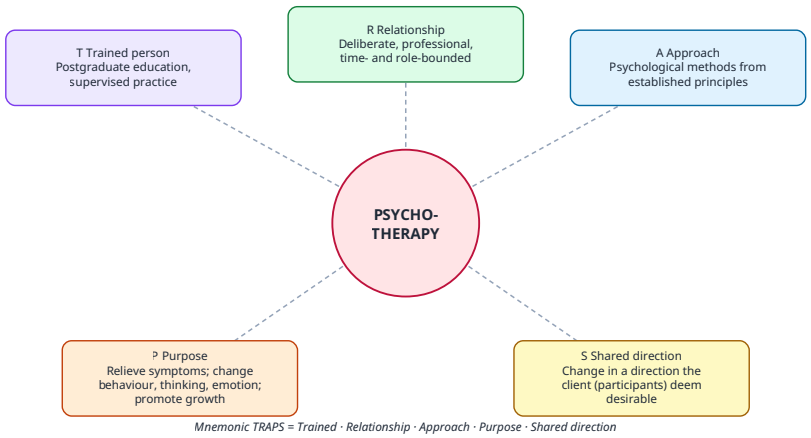


Figure 1.1 Elements common to modern definitions (Wolberg; Norcross 1990 as cited in Notes p.1–2).
Source/basis: Wolberg 2013 PDF p.29; Notes-Psychotherapies PDF p.1–2

Classic definitions

- **THEORETICAL VIEW** Wolberg (1954) defines psychotherapy as the treatment of problems of an emotional nature in which a trained person deliberately establishes a professional relationship to help a client change. [Wolberg, PDF p. 29]
- Wolberg's definition sets three explicit aims, namely to remove, modify or retard existing symptoms, to mediate disturbed patterns of behaviour (long-standing unhelpful ways of acting and relating), and to promote positive personality growth.
- **THEORETICAL VIEW** Jerome Frank (1961) describes therapy as a planned, emotionally charged (involving strong, genuine feeling), confiding interaction between a trained, socially sanctioned healer and a sufferer that aims to change the sufferer's feelings and attitudes.
- **THEORETICAL VIEW** Norcross (1990) calls it the informed and intentional application of clinical methods and interpersonal stances (the therapist's deliberate way of relating to the client) drawn from established psychological principles to help people change.
- **THEORETICAL VIEW** Strupp views psychotherapy as a learning process (change achieved through new experience and understanding, not through drugs) in which a trained person uses the relationship to help the patient overcome distress and function better.
- Across these authorities the emphasis shifts but the core is constant, because each insists on a trained helper, a client in distress, a psychological method and a goal of desirable, agreed change.

ELEMENTS COMMON TO ALL DEFINITIONS

- A trained, professional therapist (socially sanctioned).
- A help-seeking client in distress.
- A psychological (not pharmacological / surgical) method.
- A planned, systematic, theory-based process.
- A confiding, therapeutic relationship (alliance).
- The aim of change: symptom relief, behaviour change and growth.

Common elements in sentences (exam form)

- Every definition requires a trained, socially sanctioned therapist, meaning a professional whose education and registration give them both the competence and the public authority to treat.
- Every definition assumes a help-seeking client in distress, because psychotherapy is a voluntary treatment entered into by someone who recognises a problem and wants help with it.
- The method is psychological rather than pharmacological or surgical (using medication or physical operations), so change is pursued through talking, relationship and new learning instead of through the body.

- The process is planned, systematic and theory-based, which distinguishes professional therapy from ordinary advice, friendship or informal emotional support.
- A confiding therapeutic relationship, often called the alliance (the bond and agreement on goals and tasks between therapist and client), is the vehicle through which techniques take effect.
- The shared aim is change the client endorses, whether that is symptom relief, altered behaviour or personal growth, so direction is negotiated rather than imposed.

Wolberg's three aims

- Wolberg's first aim is to remove, modify or retard existing symptoms, that is to reduce the client's current suffering such as anxiety, low mood or compulsive behaviour.
- His second aim is to mediate, meaning to correct, disturbed patterns of behaviour that keep the client stuck in unhelpful ways of acting and relating.
- His third aim is to promote positive personality growth and development, so that therapy builds strengths and maturity rather than only subtracting symptoms. [Wolberg, PDF p.29]

Psychotherapy vs counselling vs medical treatment

Overlapping but distinguishable helping activities (difference is mostly of degree).

Feature	Counselling	Psychotherapy	Medical treatment
Focus	Normal developmental, situational, adjustment problems	Deeper, enduring or pathological problems; personality reconstruction	Diagnosable illness via biological route
Time frame	Present- and problem-focused, shorter	Often longer, may explore past	Variable
Method	Guidance, support, problem solving	Relationship + theory-based technique	Drugs, procedures
Overlap	Large overlap with psychotherapy	Often combined with medication	Often combined with psychotherapy (e.g. depression, psychosis)

- Counselling usually deals with normal developmental, situational and adjustment problems (difficulty coping with an identifiable life change or stress), is more present- and problem-focused, and tends to be shorter in duration.
- Psychotherapy addresses deeper, more enduring or pathological problems and personality reconstruction (reshaping long-standing traits and ways of relating), usually runs longer, and may explore the client's past as well as the present.
- The counselling-psychotherapy boundary is one of degree rather than a sharp line, because the methods overlap heavily and many practitioners use the two terms almost interchangeably in practice.
- Psychotherapy works through communication, relationship and learning rather than drugs or surgery, which distinguishes it from the biological route (treatment by medication, physical procedures or other bodily means) of medical care.
- Psychological and medical treatments are frequently combined, for example pairing antidepressant medication with CBT for depression or antipsychotics with psychosocial therapy for psychosis.
- **THEORETICAL VIEW** Because the distinction is mostly of degree, examiners expect you to name the overlap first, then separate the activities by focus, time frame and method rather than by a rigid wall.

MNEMONIC

TRAPS

Trained person, Relationship (confiding, professional), Approach (psychological, theory-based), Purpose (symptom relief and behaviour change), Shared direction (change the client endorses) — the five elements common to all definitions.

EXAM POINTER

- If asked to “define psychotherapy”, give one classic definition (Wolberg or Frank), then list the five common elements (TRAPS).
- If asked to “distinguish”, use the table rows: focus, time frame, method and overlap.

ANSWER SKELETON

- Open with one classic definition (Wolberg or Frank).
- List the six elements common to all definitions.
- State Wolberg's three aims.
- Distinguish psychotherapy from counselling and medical treatment (table rows).
- Close: the distinction is one of degree; methods often combine.

PRACTICE QUESTIONS

- Define psychotherapy and bring out its essential features.
- Differentiate psychotherapy from counselling.
- Discuss Wolberg's three aims of psychotherapy.

2 Objectives (goals) of psychotherapy

Objectives run from immediate symptom relief to long-term growth; most therapies pursue several at once, weighted by the model and the client's needs.

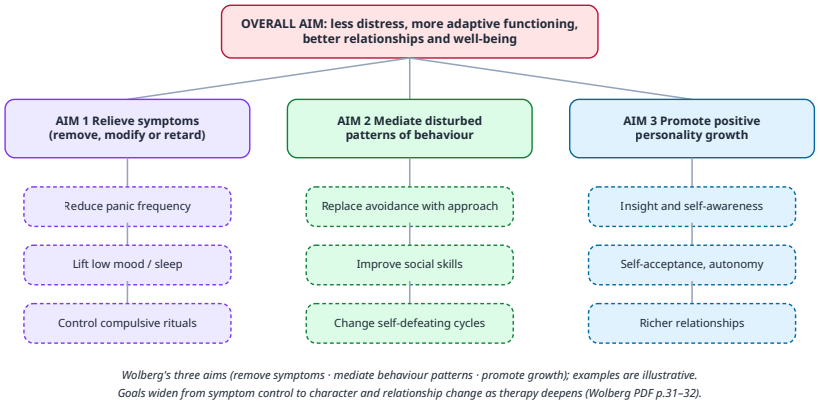


Figure 2.1 From overall aim to specific objectives.
Source/basis: Wolberg 2013 PDF p.29, 31–32

- The first objective is to relieve the client's symptoms and distress, such as anxiety, low mood, obsessions (unwanted, intrusive and repetitive thoughts) or sleep disturbance, so that daily suffering is reduced.
- A second objective is to modify maladaptive (unhelpful and self-defeating) behaviour, thoughts and emotions, replacing patterns that maintain the problem with more adaptive and flexible responses.
- Many therapies aim to help the client gain insight (an accurate understanding of the origins and meaning of one's difficulties), which is especially central to psychodynamic work and supports lasting change.
- Therapy frequently seeks to improve interpersonal relationships and social functioning, because distress is often created and maintained within the client's close relationships and wider social roles.
- A further objective is to strengthen the client's coping, problem-solving and adaptive defences (healthy ways of managing stress and difficult feelings), while raising self-esteem and self-efficacy (the belief that one can succeed at a task).
- Some objectives focus on resolving inner conflicts and past trauma and on fostering emotional regulation (the capacity to notice, tolerate and manage one's feelings) rather than being overwhelmed by them.
- At the growth end, therapy promotes personal development, self-actualisation (realising one's fuller potential and becoming more fully oneself) and a renewed sense of meaning and purpose in living.
- A long-term objective is to prevent relapse and improve quality of life, and in severe or enduring illness to maintain functioning in the community and prevent avoidable hospitalisation.
- Objectives are always negotiated with the client rather than imposed, so that the ends pursued are ones the client genuinely values and is motivated to work towards.
- Because objectives compete for limited time, the therapist orders them, placing safety and symptom stabilisation before the deeper work of insight, relationships and personal growth.

OUTCOME VS PROCESS GOALS

- Outcome goals = the end the client wants (less panic, better sleep).
- Process goals = the in-therapy means (insight, alliance, skills practice).
- Both are negotiated and revisited as therapy proceeds.

How objectives differ by model

- In psychodynamic therapy the central objective is insight (understanding how past relationships and unconscious conflict shape present difficulties), together with working through and the development of more mature defences.
- In cognitive-behavioural therapy the objectives are symptom reduction, the changing of unhelpful thoughts and behaviours, the learning of new coping skills, and the prevention of relapse.
- In humanistic therapy the objectives are self-actualisation (realising one's fuller potential), congruence (being genuine and consistent between inner experience and outward expression), self-acceptance, autonomy and a sense of meaning.
- In systemic or family therapy the objectives shift from the individual to the system, aiming to change repeating interaction patterns, clarify boundaries and improve relationships.
- Because models weight objectives differently, the therapist chooses and sequences goals in line with the client's problem, preferences and the model being used rather than applying one fixed list.

ILLUSTRATIVE EXAMPLE (FICTIONAL)

ILLUSTRATIVE EXAMPLE A client with panic names her outcome goal as travelling to work without a panic attack. The therapist adds process goals of building the alliance, practising interoceptive exposure and learning to tolerate anxiety, and together they order safety and symptom relief before deeper relationship work. Details invented.

ORDERING OBJECTIVES IN PRACTICE

- Safety first: manage risk (suicidality, harm, abuse) before other goals.
- Then stabilise symptoms and distress, and build the alliance and coping skills.
- Then address underlying patterns, relationships and growth.
- Finally consolidate gains and plan relapse prevention and maintenance.

MNEMONIC

RICHE-S

Relieve symptoms, Insight, Change maladaptive patterns, Healthy relationships, Enhance coping and self-esteem, Self-actualisation and growth — the ladder of objectives from relief to growth.

GUIDELINE WHO frames mental-health care around functioning, participation and rights, not symptom removal alone — consistent with objectives such as social functioning and quality of life. [WHO]

ANSWER SKELETON

- Group objectives as symptom relief, change, insight, relationships, growth, prevention.
- Distinguish outcome from process goals.
- Note how objectives vary by model (psychodynamic, CBT, humanistic, systemic).

PRACTICE QUESTIONS

- Describe the objectives of psychotherapy.
- How do the goals of psychotherapy differ across the major schools?

3 The process of psychotherapy at a glance

Therapy moves through recognisable stages; the loop of assess » formulate » plan » intervene » review repeats until goals are met.

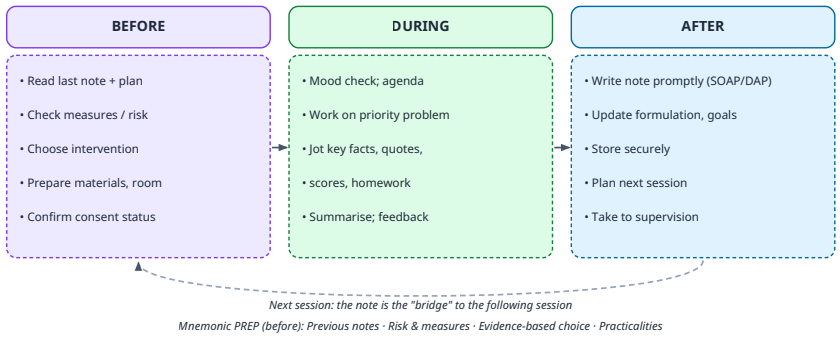


Figure 7.1 The planning and recording cycle.
Source/basis: Beck 2011 PDF p.122-123, 143; Jongsma 2014 PDF p.24-26; APA Record Keeping Guidelines 2007

1. Referral / help-seeking and first contact.
2. Intake, assessment and risk screening.
3. Case formulation (a working hypothesis of the problem).
4. Contracting, goal-setting and treatment planning.
5. Active intervention and working through.
6. Monitoring progress with repeated measures.
7. Consolidation, relapse prevention and termination.
8. Follow-up / review.

READING THE MAP

- The early steps (intake, formulation, contracting) belong to the beginning phase.
- Intervention and progress-monitoring repeat through the middle phase.
- Consolidation, relapse prevention and termination close the end phase; follow-up checks maintenance.
- The cycle is iterative: new information re-opens formulation and re-sets goals.

4 Issues related to the training of professional therapists

Training defines who may practise, what competencies are required, and how the public is protected from unqualified practice.

Who may practise (India)

- **BINDING LAW** The Rehabilitation Council of India (RCI) Act, 1992 (amended 2000) regulates training, registration and conduct of rehabilitation professionals, including clinical psychologists. [RCI Act 1992]
- **BINDING LAW** Only persons enrolled on the Central Rehabilitation Register (CRR) may practise; practising without registration is an offence. [RCI Act 1992]
- The RCI-recognised M.Phil. in Clinical Psychology (two years, with supervised clinical practice) is the entry qualification.
- **BINDING LAW** The Mental Healthcare Act (MHCA), 2017 recognises the clinical psychologist (RCI-recognised qualification) as a mental health professional. [MHCA 2017]
- **SECONDARY SOURCE** The IACP (Indian Association of Clinical Psychologists) is a professional association that advocates on training and professional identity; its code of conduct could not be primary-sourced here, so no clause numbers are quoted.

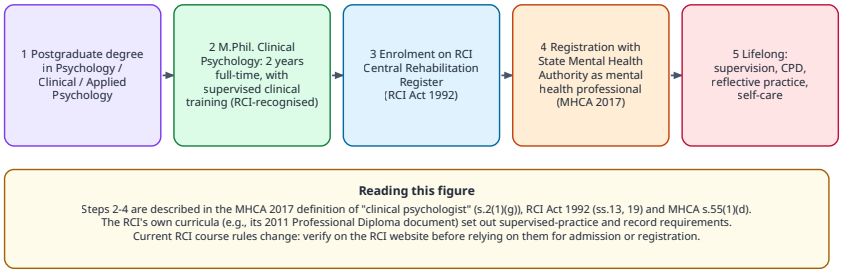


Figure 3.1 Pathway to practise as a clinical psychologist (India).
Source/basis: MHCA 2017 Gazette text; RCI Act 1992 text; RCI Professional Diploma document 2011

Components of training

- Theoretical knowledge:
  Psychopathology and classification; theories of psychotherapy; principles of assessment and psychometrics.
- Supervised clinical practice:
  Direct observation, audio/video review, case conferences and live supervision under qualified supervisors.
  Graded responsibility, from observing to co-therapy to supervised independent work.
- Personal therapy or self-awareness work (to manage countertransference and blind spots).
- Learning ethics and law and how to apply them in practice.
- Research literacy and the skills to practise evidence-based care.
- Cultural competence and working with diversity.
- Lifelong learning and continuing professional education.

Competencies expected

- Relationship (alliance) skills.
- Assessment and case formulation.
- Intervention skills in at least one major model.
- Ethical decision making and record keeping.
- Working with diversity; use of supervision; reflective practice.

APA STANDARDS ON COMPETENCE & TRAINING

- PROFESSIONAL CODE APA Ethics Code Std 2.01 — practise only within boundaries of competence. [APA 2017, Std 2.01]
- PROFESSIONAL CODE Std 2.03 — maintain competence; Std 2.06 — manage personal problems that could impair work. [APA 2017]
- PROFESSIONAL CODE Std 10.01(c) — when a trainee provides therapy, the client is told the trainee is supervised and given the supervisor's name. [APA 2017, Std 10.01(c)]

Issues and challenges

- There is wide variation in the quality, duration and supervision of training across institutions, and a shortage of trained supervisors limits how many competent therapists can be produced each year.
- Indian training relies heavily on Western models that may not fit local realities, so there is a pressing need for culturally adapted and indigenous (developed from and suited to the local culture) approaches.
- It is debated whether personal therapy should be compulsory for trainees, the argument being that it helps them manage countertransference (the therapist's own feelings stirred up by the client) and their personal blind spots.
- Programmes must act as gatekeepers, judging suitability on empathy, maturity and integrity, and must be willing to remediate or, in the last resort, dismiss trainees who are unfit to practise.
- A training-versus-licensing gap allows untrained persons to practise, so protecting the public from quacks (unqualified practitioners posing as therapists) through registration and enforcement is an ongoing concern.
- Therapist stress, burnout, vicarious traumatisation (distress absorbed from repeatedly hearing clients' trauma) and compassion fatigue are real risks that make self-care and peer support part of competent training.
- Boundaries and dual relationships (having more than one kind of relationship with the same person) are especially tricky in trainee-supervisor pairings and in small communities where people are already acquainted.
- Training must now include tele-therapy and its particular ethics, since remote delivery raises fresh questions about privacy, consent, jurisdiction and crisis management.
- Throughout their careers therapists must practise within the limits of their competence and recognise when a case should be referred to a more suitable or more senior colleague.
- Staying evidence-based requires therapists to integrate new research into their continuing professional development rather than relying indefinitely on what they first learned.

MNEMONIC

TRAIN

Theory, Relationship skills, Assessment, Intervention, iNtegrity/ethics — the core domains a programme must build.

WHO: TASK-SHARING AND SCALING TRAINING

GUIDELINE The WHO mhGAP Intervention Guide and Problem Management Plus (PM+) support training non-specialist and lay providers to deliver brief psychological care in low-resource settings — relevant to the training-supply gap in India. [WHO mhGAP]

Supervision and self-care

- THEORETICAL VIEW Supervision protects the client and develops the trainee: direct observation, audio/video review, case discussion and reflective feedback. [Gabbard, PDF p.192-193]
- Supervisees must disclose difficulties honestly; supervisors hold responsibility for the client's care during training.
- Self-care (peer support, personal therapy, workload limits) guards against burnout, vicarious trauma and compassion fatigue.

GUIDELINE APA Telepsychology Guidelines ask therapists to be competent in the technology they use and to manage the extra confidentiality and jurisdiction issues of tele-therapy — now a training need. [APA Telepsychology]

ANSWER SKELETON

- Who may practise in India: RCI Act, CRR, M.Phil.; MHCA recognition.
- Components of training and expected competencies.
- Issues: quality variation, cultural adaptation, gate-keeping, self-care.
- Role of supervision; WHO task-sharing for the supply gap.

PRACTICE QUESTIONS

- Discuss the issues involved in training professional therapists.
- What competencies should the training of a clinical psychologist develop?

5 Ethical issues in therapy work

Ethics sets the higher, aspirational standard of conduct (law sets the enforceable minimum). The APA's five general principles are the most widely taught framework.

APA five general principles (2017)

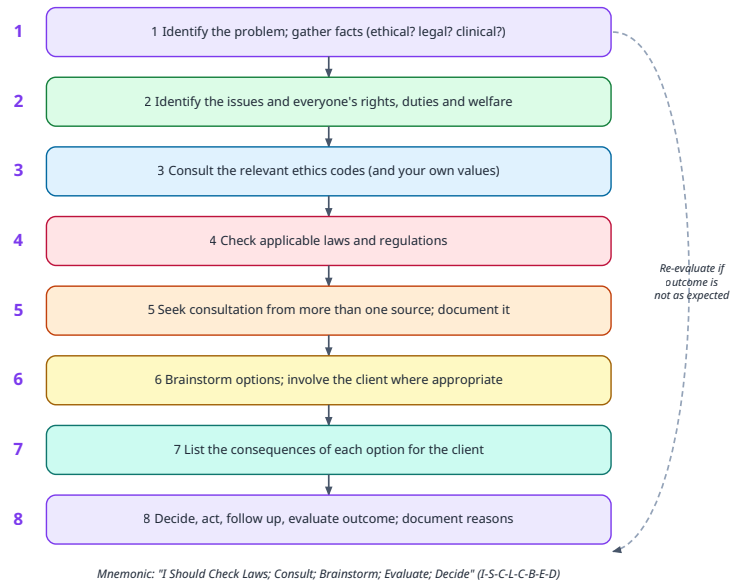


Figure 4.2 Ethical decision-making steps (after Corey et al., 2024).
Source/basis: Corey 2024 PDF p.74–75

APA (2017) General Principles A–E. [APA 2017]

Principle	Meaning
A. Beneficence & non-maleficence	Do good; avoid harm; weigh benefits against risks.
B. Fidelity & responsibility	Trust, professional and scientific responsibility, accountability.
C. Integrity	Honesty, accuracy, truthfulness; no deception or exaggerated claims.
D. Justice	Fairness, equal access to quality services; no discrimination.
E. Respect for rights & dignity	Privacy, confidentiality, autonomy, self-determination.

PROFESSIONAL CODE These five principles are aspirational; APA's numbered Standards (e.g. 2.01, 3.04, 3.05, 3.10, 4.01–4.07) are enforceable for members. The RCI Code of Conduct reflects similar duties. [APA 2017]

Major ethical issues

- Competence requires the therapist to practise only within the limits of their training and experience, to refer problems that fall outside that scope, and to keep knowledge and skills current. [APA 2017, Std 2.01]
- Informed consent must be obtained before any assessment or treatment and treated as an ongoing process rather than a single signature, so the client agrees freely and understands what is proposed (see Section 8).
- Confidentiality means private information is shared only with the client's consent, and the therapist must explain its limits at the outset so trust is not later broken by surprise. [APA 2017, Std 4.01-4.02]
- Confidentiality has recognised exceptions, namely serious risk to the client or an identifiable other, suspected abuse of a child, and a lawful court order (a direction from a court that legally compels disclosure).
- Avoiding harmful dual (multiple) relationships (having a second role with a client, such as friend, employer or lover, alongside being their therapist) is essential, so social, business, financial and sexual ties with clients are prohibited. [APA 2017, Std 3.05-3.08]
- Sexual intimacy with a current client is absolutely forbidden, and with a former client it is prohibited for a specified period, because the power imbalance makes genuine consent doubtful and the risk of harm high.
- Boundary issues such as gifts, self-disclosure (the therapist revealing personal information), physical touch and social-media contact must be managed thoughtfully so that the work stays focused on the client's needs.

- Fees and contracts must be clear, honest and agreed in advance, and the therapist must never exploit the client's dependence for financial, personal or other advantage.
- Record keeping and data security are ethical duties, because inaccurate, careless or insecurely stored records can harm the client and breach confidentiality.
- Termination and referral must be handled so that clients are not abandoned, which means giving notice, summarising progress and arranging a proper handover to continuing care.
- Cultural sensitivity requires the therapist to avoid bias linked to gender, caste, religion, sexuality, disability or socio-economic status and to adapt methods to the client's context.
- Seeking supervision and consultation when facing an ethical dilemma is itself an ethical duty, since a second professional view guards against blind spots and self-serving reasoning.
- Advertising and public claims must be truthful, so the therapist makes no false or unsubstantiated (not backed by evidence) promises about cure rates or guaranteed outcomes.
- Research with clients carries its own duties of informed consent, avoidance of harm, confidentiality and prior approval by an ethics committee before the study begins.

ETHICAL DECISION-MAKING STEPS

- Identify the problem and the people affected.
- Consult codes and law.
- Consult supervisor / colleagues.
- Consider options and consequences.
- Decide and act.
- Document, then evaluate the outcome.

Limits of confidentiality

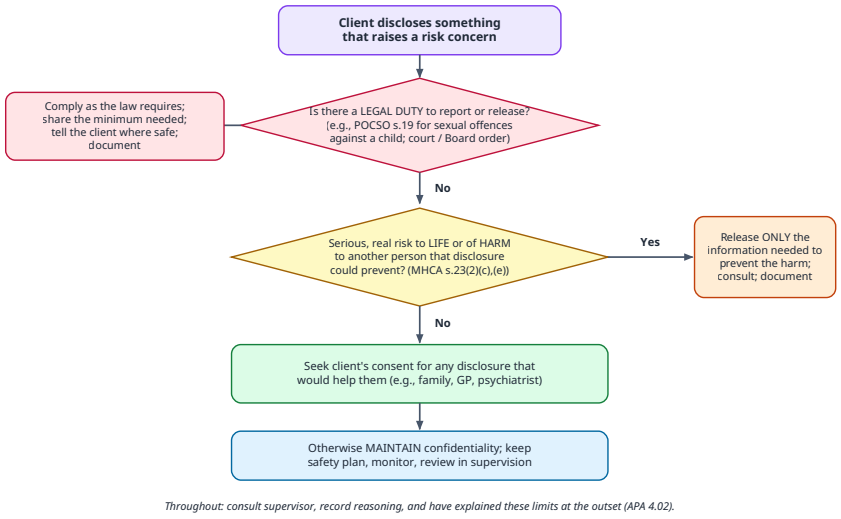


Figure 4.3 Confidentiality: when to maintain and when to disclose (teaching schematic, not legal advice).
Source/basis: MHCA 2017 s.23(2); POCSO 2012 s.19; APA Ethics Code Std 4.01-4.05; Corey 2024 PDF p.76-77

CONFIDENTIALITY DECISION AS STEPS (DOCX VIEW)

- Step 1: If a legal duty applies (for example POCSO s.19, or a court or Board order), comply, share only the minimum needed, and document.
- Step 2: Else if there is a serious, real risk to life or of harm to another that disclosure could prevent, release only what is needed to prevent the harm.
- Step 3: Else seek the client's consent for any disclosure that would help them, such as involving family or a psychiatrist.
- Step 4: Otherwise maintain confidentiality, keep a safety plan, monitor, and review the situation in supervision.

- **BINDING LAW** Tarasoff v. Regents of the University of California (1976) established a duty to warn and protect identifiable third parties from a client's serious threats of violence. [Tarasoff 1976]
- **BINDING LAW** MHCA 2017 protects confidentiality of information about a person's mental illness and treatment; disclosure is allowed with consent, for the treatment, for safety, or by court order. [MHCA 2017, s.23]
- **BINDING LAW** POCSO Act, 2012 makes reporting of known or suspected child sexual abuse mandatory. [POCSO 2012, s.19]

- BINDING LAW The right to privacy is a fundamental right (Puttaswamy v. Union of India, 2017). [Puttaswamy 2017]

ILLUSTRATIVE EXAMPLE (FICTIONAL)

ILLUSTRATIVE EXAMPLE A client states a specific, credible plan to seriously harm a named person. The therapist consults the supervisor, documents the reasoning, warns the identifiable third party and informs authorities, and limits disclosure to what safety requires. Names and details here are invented.

SECONDARY SOURCE The IACP code of conduct for its members is reported to stress competence, responsibility, client welfare and integrity — broadly matching the APA principles above. The primary code could not be read here, so no clause numbers are given.

How ethics bodies map onto this topic

- PROFESSIONAL CODE APA Ethics Code (2017): five general principles plus enforceable Standards (competence 2.01, avoiding harm 3.04, multiple relationships 3.05, informed consent 3.10, confidentiality 4.01–4.07). [APA 2017]
- BINDING LAW RCI Act s.21 empowers the Council to prescribe standards of professional conduct and a code of ethics; breach can lead to removal from the register. [RCI Act 1992, s.21]
- SECONDARY SOURCE IACP: members' code of conduct (secondary summary only).
- GUIDELINE BPS (UK) states four principles: respect, competence, responsibility, integrity — a useful cross-check.

MNEMONIC

FIBRE (A-E)

Fidelity and responsibility (B), Integrity (C), Beneficence and non-maleficence (A), Respect for rights and dignity (E), and justicE / Equity (D) — one word recalling the APA's five general principles A to E.

ANSWER SKELETON

- Define ethics vs law (aspirational vs enforceable minimum).
- State APA's five general principles.
- List the major ethical issues (competence, consent, confidentiality, boundaries).
- Give the ethical decision-making steps; add the Tarasoff/POCSO limits of confidentiality.

PRACTICE QUESTIONS

- Discuss the ethical issues involved in psychotherapy.
- Explain the limits of confidentiality with reference to law.
- Describe a model of ethical decision-making.

6 Legal issues in therapy work (India)

Indian psychotherapy practice is governed by a statutory registration law, a rights-based mental-health law, child-protection law, disability law, consumer law and tele-practice guidelines.

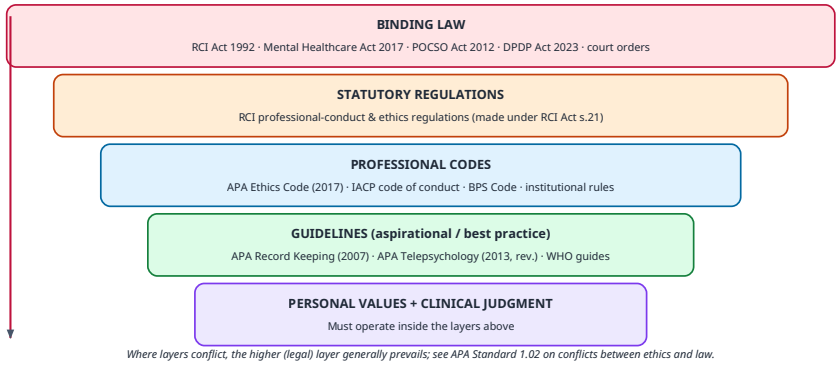


Figure 4.1 Binding force decreases down the stack; ethical reasoning is still needed at every level.
Source/basis: RCI Act 1992 s.21; APA Ethics Code 2017 Std 1.02; Notes p.9–10

READING THE AUTHORITY STACK (DOCX STEP VIEW)

- Step 1: Binding law — RCI Act 1992, MHCA 2017, POCSO 2012 and court orders sit at the top and must always be obeyed.
- Step 2: Statutory regulations — the RCI's conduct and ethics regulations made under s.21 apply next.
- Step 3: Professional codes — APA Ethics Code, the IACP code and institutional rules bind members.
- Step 4: Guidelines — aspirational best practice such as APA record-keeping and telepsychology guidance.
- Step 5: Personal values and clinical judgement operate only inside all the layers above; where they conflict, the higher legal layer prevails.

- **BINDING LAW** The RCI Act 1992 regulates the training, registration and conduct of rehabilitation professionals including clinical psychologists, and practising without registration on the Central Rehabilitation Register is a punishable offence. [RCI Act 1992]
- **BINDING LAW** The Mental Healthcare Act 2017 is a rights-based law that reframes mental-health care around the dignity, autonomy and legal entitlements of the person rather than around institutional control. [MHCA 2017]
- Under the MHCA every person with mental illness has rights to affordable quality care, community living, confidentiality, access to their own records, equality and freedom from cruel or degrading treatment (treatment that humiliates or demeans the person).
- The MHCA lets a person make an advance directive (a statement, made while well, of how one wishes to be treated during a future crisis) and name a nominated representative (a trusted person chosen to take treatment decisions on one's behalf).
- The Act presumes that every adult has the capacity to make their own treatment decisions unless the contrary is properly established, protecting clients from being overridden without good reason.
- **BINDING LAW** Section 115 presumes that a person who attempts suicide is under severe stress and is not to be punished, and it places a duty on government to provide care and rehabilitation. [MHCA 2017, s.115]
- The MHCA restricts ECT (electroconvulsive therapy, a medical treatment passing a brief electric current through the brain), barring it without anaesthesia and muscle relaxant and barring it for minors, and limits the use of restraints and sterilisation.
- **BINDING LAW** The Rights of Persons with Disabilities Act 2016 covers mental illness and intellectual disability and underpins non-discrimination, reasonable accommodation and access to services.
- **BINDING LAW** The POCSO Act 2012 makes reporting of known or suspected child sexual abuse mandatory, and the Juvenile Justice Act governs the care and protection of children. [POCSO 2012, s.19]
- **BINDING LAW** The Consumer Protection Act and the civil and criminal law on negligence (failing to take the care a reasonably competent professional would take, causing harm) expose therapists to malpractice claims, and courts may require records or testimony.
- **GUIDELINE** For tele-mental health the Telemedicine Practice Guidelines 2020 and guidance on tele-psychiatry apply, adding duties around identity verification, consent and secure technology.
- **BINDING LAW** The right to privacy is a fundamental right after Puttaswamy (2017), which strengthens the legal basis for confidentiality and careful handling of client data. [Puttaswamy 2017]

LEGAL VS ETHICAL

- Law sets the minimum enforceable standard; breach can mean penalty or loss of registration.
- Ethics sets the higher aspirational standard of conduct.
- Therapist's legal duties: valid consent, confidentiality, competent practice, accurate records, reporting obligations, no exploitation, referral when needed.

- Law and ethics overlap but are not identical, because the law states the enforceable floor below which conduct becomes punishable while ethics describes the higher standard a good therapist aspires to.
- Where a legal duty and an ethical value appear to clash, the therapist clarifies the true legal obligation, seeks consultation, and documents the reasoning rather than acting on assumption.
- A therapist's core legal duties include obtaining valid consent, protecting confidentiality within its limits, practising competently, keeping accurate records and meeting mandatory-reporting obligations.

MNEMONIC

RM-PCDT

RCI Act, MHCA 2017, POCSO 2012, Consumer/negligence law, Disability (RPwD 2016) Act, Telemedicine guidelines — the chain of Indian legal sources a therapist must respect.

ANSWER SKELETON

- List the main Indian statutes (RCI, MHCA, RPwD, POCSO, consumer/negligence law).
- Expand MHCA 2017 rights and Section 115.
- State the therapist's legal duties; contrast legal with ethical standards.

PRACTICE QUESTIONS

- Describe the legal issues involved in therapy work in India.
- Outline the key provisions of the Mental Healthcare Act, 2017 relevant to a therapist.

7 Rights and responsibilities in psychotherapy

Therapy is a contract: both parties hold rights and duties. Clarifying them at the start supports the alliance and informed consent.

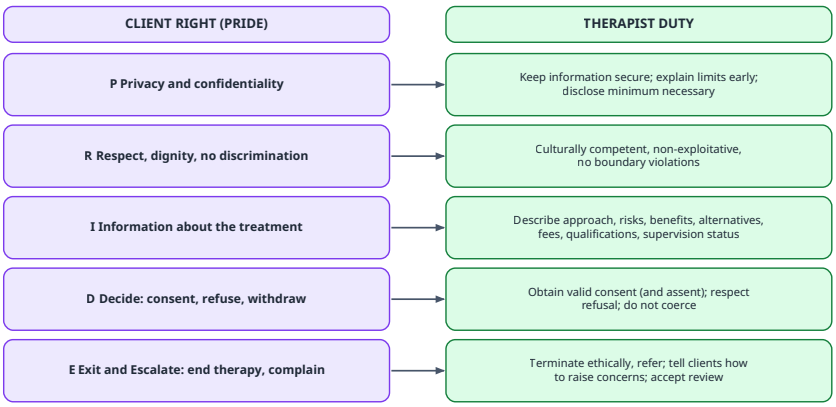


Figure 5.1 Rights imply duties. Mnemonic **PRIDE**.
Source/basis: MHCA 2017 ss.4, 20-25; APA Ethics Code Std 3.10, 4.01-4.02, 10.01, 10.10; Corey 2024 PDF p.75

RIGHTS MAP TO DUTIES (DOCX STEP VIEW)

- Privacy and confidentiality (client right) -> therapist keeps information secure and discloses only the minimum necessary.
- Respect and non-discrimination -> therapist practises in a culturally competent, non-exploitative way with no boundary violations.
- Information about treatment -> therapist describes approach, risks, benefits, alternatives, fees and qualifications.
- Decide (consent, refuse, withdraw) -> therapist obtains valid consent and assent and never coerces the client.
- Exit and Escalate (end therapy, complain) -> therapist terminates ethically, refers, and tells clients how to raise concerns.

MNEMONIC

PRIDE

Privacy/confidentiality, Respect and dignity, Information about treatment, Decide (consent, refuse, withdraw), Exit and Escalate (end therapy, complain) — the client's core rights, each mirrored by a therapist duty.

Client rights and client responsibilities.

Client rights	Client responsibilities
Informed consent; know the therapist's training, approach and fees	Attend sessions regularly and on time; give notice of cancellation
Confidentiality (with known limits)	Be honest and open about problems, history and progress
Be treated with respect and dignity, free from discrimination	Actively participate and complete agreed homework
Know the nature, expected duration and alternatives of treatment	Pay agreed fees; follow the agreed contract
Refuse or withdraw from therapy at any time	Respect the therapist's boundaries
Access own records; a second opinion; make complaints	Inform the therapist about other treatments, medicines, risks

Therapist responsibilities.

Therapist responsibilities
Competence; keep within limits of training; refer when appropriate
Obtain informed consent; maintain confidentiality

Maintain boundaries; avoid dual and exploitative relationships
Keep accurate records; protect them; follow reporting laws
Respect diversity; use culturally appropriate methods
Continue supervision and learning; practise self-care
Terminate appropriately; ensure continuity of care

Expanded in sentences (exam form)

- Psychotherapy is best understood as a therapeutic contract (an explicit mutual agreement about aims, methods, fees and boundaries) in which both parties knowingly accept rights and matching responsibilities.
- The client has a right to informed consent, which means knowing the therapist's training, approach, likely duration, fees and the alternatives before agreeing to proceed.
- The client has a right to confidentiality subject to its known limits, so disclosures made in therapy are protected except where law or serious risk requires a narrow breach.
- The client has a right to be treated with respect and dignity and to be free from discrimination based on gender, caste, religion, disability, sexuality or socio-economic status.
- The client retains the right to refuse or withdraw from therapy at any time without penalty, which keeps participation genuinely voluntary throughout the work.
- The client has a right to access their own records, to seek a second opinion and to make a complaint through a recognised grievance route if dissatisfied.
- In return the client is responsible for attending regularly, giving notice of cancellation, being honest about history and progress, and completing agreed homework (tasks practised between sessions to extend therapy into daily life).
- The therapist is responsible for working only within their competence, obtaining valid consent, protecting confidentiality and maintaining clear professional boundaries at all times.
- The therapist is further responsible for keeping accurate records, respecting diversity, using supervision, practising self-care, and ensuring continuity of care when therapy ends.
- The therapist also holds rights, including the right to set a safe working frame, to be free from abuse or harassment, and to decline work that cannot be conducted ethically or competently.

BINDING LAW Many client rights are also statutory under MHCA 2017 ss.18–25 (information, confidentiality, access to records, complaints). [MHCA 2017]

WHY THIS MATTERS

- Spelling out rights and duties at the first session operationalises informed consent and the therapeutic contract.
- GUIDELINE It also reflects a rights-based model of care consistent with the MHCA 2017 and WHO guidance on dignity and participation.
- Clear mutual expectations reduce boundary problems, missed-session disputes and premature drop-out.

Therapist rights too

- To set and maintain professional boundaries and a safe working frame.
- To be treated with respect and to work free from abuse, threats or harassment.
- To decline or terminate work that is outside competence or that cannot be conducted ethically, with appropriate referral.
- To fair remuneration under the agreed contract.

ANSWER SKELETON

- Introduce therapy as a contract of mutual rights and duties.
- Give client rights and responsibilities (table).
- Give therapist responsibilities.
- Link rights to MHCA 2017 ss.18–25 and a rights-based model of care.

PRACTICE QUESTIONS

- Discuss the rights and responsibilities of the client and therapist in psychotherapy.

8 Consent and assent (including minors)

Informed consent is the client's voluntary agreement to assessment or therapy after receiving and understanding adequate information. It is a process, not a one-time signature.

Elements of valid consent

- Capacity (competence): ability to understand, appreciate, reason and express a choice.
- Disclosure of information:
  - Purpose and nature of therapy, methods, expected benefits.
  - Possible risks and discomfort; alternatives (including no treatment).
  - Duration, fees, confidentiality and its limits.
  - Recording or observation, supervision, therapist's qualifications.
  - The right to withdraw without penalty.
- Comprehension: simple, understandable language; check understanding.
- Voluntariness: free from coercion or undue pressure.
- Documentation: written (or recorded) consent, revisited when the plan changes.

PROFESSIONAL CODE APA Std 3.10 (informed consent) and 10.01 (consent to therapy) set out these duties; Std 3.10(b) lists four duties when a person cannot consent. [APA 2017, Std 3.10 & 10.01]

Elements in sentences (exam form)

- Valid consent first requires capacity (the mental ability to understand information, weigh it and communicate a choice), which is presumed in adults unless a specific impairment is shown.
- The therapist must disclose adequate information, covering the nature and purpose of therapy, its expected benefits, foreseeable risks and the realistic alternatives including no treatment.
- Disclosure must also state the likely duration, the fees, the limits of confidentiality, any recording or supervision, and the client's right to withdraw without penalty.
- Comprehension matters as much as disclosure, so the therapist uses plain language, avoids jargon, and checks understanding rather than assuming the client has grasped the information.
- Consent must be voluntary, meaning it is given freely and without coercion, undue pressure or inducement from the therapist, family members or an institution.
- Consent is an ongoing process rather than a one-time signature, so it is revisited whenever the treatment plan, the risks or the therapeutic methods change materially.
- Consent should normally be documented in writing, and the record should note what was explained, what the client agreed to, and that their questions were answered.
- For a minor, legal consent is given by the parent or guardian while the child's own assent (the child's affirmative agreement, sought to the extent of their developing understanding) is sought alongside it.

Special issues

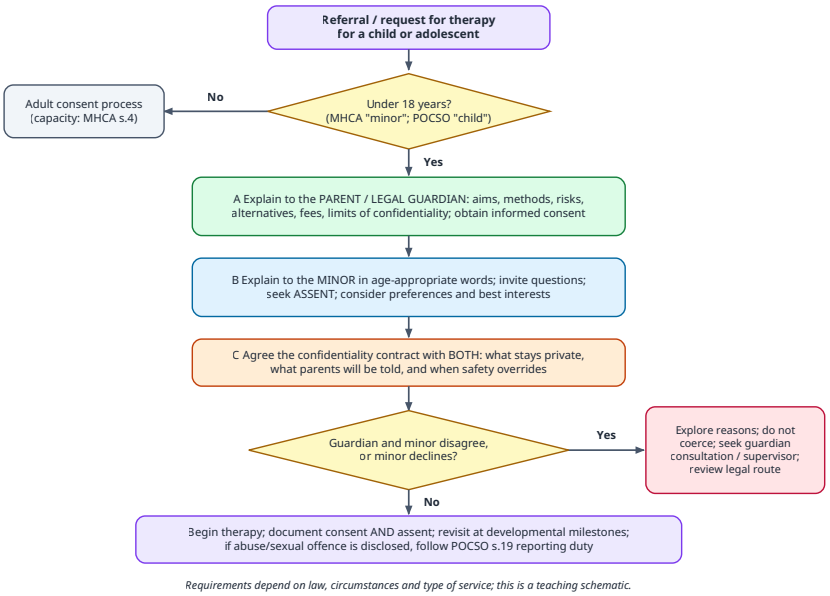


Figure 6.1 Practical flow for consent (guardian) and assent (minor).
Source/basis: APA Ethics Code Std 3.10(b), 4.02; MHCA 2017 ss.2, 4, 15; POCSO 2012 ss.2(d), 19; Case Approach PDF p.39

CONSENT-ASSENT FLOW AS STEPS (DOCX VIEW)

- Step 1: Check whether the client is under 18 (a minor under MHCA and a child under POCSO); if an adult, use the normal adult-capacity consent process.
- Step 2: Explain aims, methods, risks, alternatives, fees and confidentiality limits to the parent or legal guardian and obtain informed consent.
- Step 3: Explain therapy to the minor in age-appropriate words, invite questions and seek assent, weighing the minor's preferences and best interests.
- Step 4: Agree a confidentiality contract with both about what stays private, what parents are told, and when safety overrides privacy.
- Step 5: If guardian and minor disagree or the minor declines, explore reasons without coercion, consult a supervisor, and consider the legal route.
- Step 6: Begin therapy, documenting both consent and assent; if abuse is disclosed, follow the POCSO s.19 reporting duty.

MNEMONIC

CARVED

Capacity, Adequate disclosure, Reasoning/comprehension, Voluntariness, Enduring process (not one-off), Documentation — the elements that make consent valid; for minors add Assent on top of guardian consent.

- Minors (<18 years in India):
  Legal consent comes from the parent or legal guardian.
  The child's assent (affirmative agreement to the extent of developing capacity) is also sought.
  GUIDELINE ICMR 2017 guidelines generally require assent for children roughly 7–18 years, with simpler language for younger children and more weight to mature adolescents. [ICMR 2017]
  If a child refuses, explore concerns and, unless safety requires otherwise, respect it.
- Adults lacking capacity:
  BINDING LAW Involve the nominated representative or legal guardian under MHCA 2017; still seek the person's views and use the least restrictive approach. [MHCA 2017]
- Consent for recording, observation, supervision, research or publication is separate and specific; case material is anonymised.
- Emergency (immediate risk to life) can justify action without full consent, with documentation.
- Cultural context: in collectivist settings family involvement is common; still ensure the client's own consent and confidentiality.

ILLUSTRATIVE EXAMPLE (FICTIONAL)

ILLUSTRATIVE EXAMPLE A 14-year-old is brought by a parent. The therapist takes the parent's legal consent, then explains therapy to the adolescent in simple words and obtains assent, agreeing what will and will not be shared with the parent. All details invented.

CONSENT VS ASSENT

- Consent = the legally valid permission (the competent adult, or a minor's parent/guardian).
- Assent = the child's own affirmative agreement, sought to the extent of their developing capacity.
- A minor's refusal (dissent) is explored and, unless safety requires otherwise, respected.

BINDING LAW POCSO 2012 defines a “child” as a person below 18 and makes reporting of known or suspected child sexual abuse mandatory; this can override a minor's or family's wish for confidentiality. [POCSO 2012, s.19]

ANSWER SKELETON

- Define informed consent as a process, not a signature.
- List the elements (capacity, disclosure, comprehension, voluntariness, documentation).
- Explain consent vs assent and the handling of minors (ICMR 7–18).
- Note adults lacking capacity (MHCA), recording consent and emergencies.

PRACTICE QUESTIONS

- Explain informed consent and its essential elements.
- Discuss consent and assent in work with minors.

9 Planning and recording of therapy sessions

Good planning turns assessment into a formulation and a concrete treatment plan; good records support continuity, supervision and accountability.

Planning

- Intake and assessment: presenting complaints, history, risk assessment, mental status, test results and diagnosis.
- Case formulation — an integrated hypothesis that explains the problem and guides the plan:
  Predisposing factors (vulnerabilities), precipitating factors (triggers).
  Perpetuating factors (what maintains the problem), protective factors (strengths and resources).
- Treatment plan:
  Goals and the chosen therapy; frequency (usually weekly) and session length (usually 45–50 minutes).
  Expected duration, measures of progress, a plan for emergencies, and criteria for termination.
- Therapeutic frame: time, place, fee, boundaries, cancellation policy and contact between sessions.
- Session planning: prepare using previous notes; plan the aims for each session but stay responsive to the client's needs.

THEORETICAL VIEW Jongsma's Treatment Planner sets out a 6-step planning sequence (problem selection » definition » goals » objectives » interventions » diagnosis). [Jongsma, PDF p.24-26]

Planning in sentences (exam form)

- Planning begins with a thorough intake and assessment in which presenting complaints, history, risk, mental state and any test results are gathered to understand the problem fully.
- The assessment is then organised into a case formulation (an integrated working hypothesis explaining why the problem began and why it persists), which links the findings into a coherent story.
- A useful formulation weighs the four Ps (predisposing, precipitating, perpetuating and protective factors), distinguishing vulnerabilities and triggers from what maintains the problem and what protects the client.
- From the formulation flows a treatment plan stating the goals, the chosen therapy, the frequency and length of sessions, the measures of progress and the criteria for ending therapy.
- The plan also sets the therapeutic frame of time, place, fee and boundaries, and it includes a plan for emergencies so that risk is managed rather than improvised.

ILLUSTRATIVE EXAMPLE (FICTIONAL)

ILLUSTRATIVE EXAMPLE For a client with low mood, the formulation notes a family history (predisposing), a recent job loss (precipitating), social withdrawal (perpetuating) and a supportive partner (protective); the plan is weekly behavioural activation for twelve sessions with PHQ-9 monitoring. Details invented.

Structure of a typical CBT session

1. Brief check-in and mood check.
2. Bridge from the previous session.
3. Collaborative agenda setting.
4. Review of homework.
5. Work on agenda items (main therapeutic work).
6. Capsule summaries during the session.
7. Assign new homework.
8. Client feedback on the session.

THEORETICAL VIEW J. Beck describes this session structure. [Beck, PDF p.82 & 122-123]

Recording

- Types of records:
  Intake / case-history form; assessment reports and test data; the treatment plan; consent forms.
  Progress notes for each session; process notes (the therapist's private reflections, kept separate).
  Risk assessments; referral and termination summaries; correspondence.
- Formats of progress notes: SOAP (Subjective, Objective, Assessment, Plan), DAP (Data, Assessment, Plan), BIRP (Behaviour, Intervention, Response, Plan).

- Good records are timely, accurate, objective, factual, brief but complete, free of judgemental language, and signed and dated.
- Purposes: continuity of care, progress monitoring, supervision, communication with other professionals, legal/ethical evidence, research and quality improvement.
- Security and confidentiality:
  Secure storage (locked cabinets, password-protected files); restricted access; coded identifiers.
  Retention for the period required by law or institutional policy; safe disposal afterwards.
- Audio/video recording needs explicit written consent; recordings are used only for the agreed purpose (such as supervision) and destroyed as agreed.

Recording in sentences (exam form)

- Clinical records exist to support continuity of care, so that another professional, or the same therapist after a gap, can understand what has happened and continue the work safely.
- Records serve accountability, because accurate, dated and signed notes provide evidence of what was done and why if the work is later questioned clinically, ethically or legally.
- Good progress notes (the brief factual record written after each session) are timely, accurate, objective and free of judgemental language, recording what was said, observed, judged and planned.
- Several structured formats exist, including SOAP, DAP and BIRP (Behaviour, Intervention, Response, Plan), each giving a consistent skeleton so that important information is not omitted.
- Process notes (the therapist's private reflections, hunches and reactions) are kept separate from the formal clinical record and generally receive greater protection from disclosure.
- Records must be stored securely, for example in locked cabinets or password-protected files with restricted access, because a confidentiality breach can seriously harm the client.
- Records are retained for the period required by law or institutional policy and then disposed of safely, so that out-of-date files do not become a lingering privacy risk.
- **BINDING LAW** The client generally has a right to access their own records, a right made statutory in India by the Mental Healthcare Act 2017 s.25. [MHCA 2017, s.25]

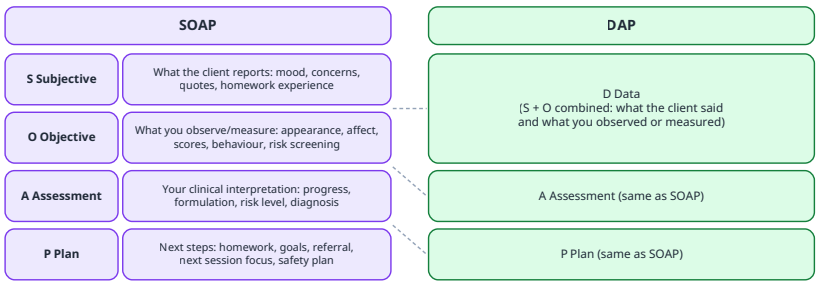


Figure 7.2 SOAP (four parts) versus DAP (three parts).
Source/basis: Standard clinical documentation formats; APA Record Keeping Guidelines 2007 (content of records)

APA GUIDANCE ON RECORDS

- **PROFESSIONAL CODE** APA Std 6.01–6.02 — create, maintain and dispose of records to protect confidentiality. [APA 2017]
- **GUIDELINE** APA Record Keeping Guidelines (2007) — content, retention and security best practice. [APA Record Keeping]
- **BINDING LAW** MHCA 2017 s.25 gives the person a right to access their own records. [MHCA 2017, s.25]

SAMPLE SOAP NOTE (FICTIONAL)

ILLUSTRATIVE EXAMPLE S: “Felt panicky on the bus twice.” O: Engaged, completed thought record. A: Reduced avoidance; PHQ-9 = 9 (down from 14). P: Continue interoceptive exposure; homework = two bus trips. Illustrative only.

MNEMONIC

SOAP

Subjective (what the client reports), Objective (what you observe/measure), Assessment (your clinical judgement), Plan (next steps).

Process notes (the therapist's private reflections) are kept separate from the formal record and are generally given more protection; progress notes form the official clinical record.

ANSWER SKELETON

- Planning: intake/assessment, formulation (4 Ps), treatment plan, frame.
- Describe a typical CBT session structure.
- Recording: types of records, SOAP/DAP/BIRP, qualities of good records, security.
- Add APA record-keeping guidance and MHCA s.25 access right.

PRACTICE QUESTIONS

- Describe the planning and recording of therapy sessions.
- What are the characteristics of good clinical records?

10 Structuring therapy and setting goals

Structure gives therapy direction, safety and efficiency; collaborative goals increase motivation and show when therapy is complete.

- Structure begins with a clear frame (the agreed boundaries of therapy: time, place, fee, length, confidentiality and contact rules), which creates the predictable, safe container within which change can take place.
- Each session is itself structured with an agenda, so that limited time is spent on the client's priorities rather than drifting without direction or purpose.
- A course of therapy is structured into three broad phases, namely a beginning, a middle and an end, each with its own characteristic tasks and emphasis.
- The beginning phase covers engagement, assessment, case formulation (a working explanation of the client's problem that guides the plan), contracting and the setting of collaborative goals.
- The middle phase is where active intervention and working through (repeatedly facing and resolving a problem until change becomes stable) occur, using the techniques of the chosen model.
- The end phase covers consolidation of gains, relapse prevention (planning ahead to recognise and manage early warning signs of a setback), termination and arrangements for follow-up.
- Regular review of progress is built into the structure, so that goals and methods are revised in light of outcome data rather than left unchanged out of habit.

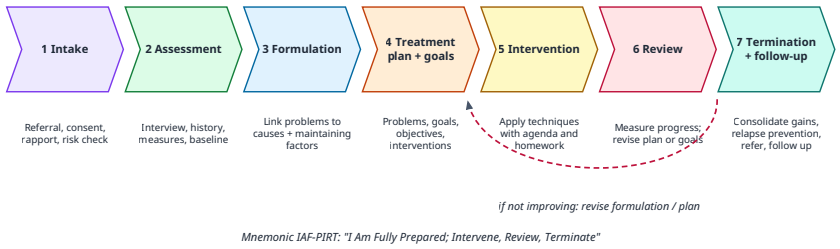


Figure 8.1 The therapy process from intake to follow-up.
Source/basis: Jongsma 2014 PDF p.24-26; Beck 2011 PDF p.70-71; RCI Diploma doc 2011 (record elements)

Goal setting

- Goals should be collaborative, meaning they are jointly negotiated with the client rather than dictated, because goals the client owns carry far more motivation and commitment.
- Goals are derived from the case formulation and expressed where possible in the client's own words, so they stay relevant to the problem and meaningful to the person.
- Well-formed goals are SMART, that is Specific, Measurable, Achievable, Realistic or Relevant, and Time-bound, which turns a vague wish into a concrete target that progress can be checked against.
- Goals are also best stated positively as something to move towards, kept under the client's own control, and ordered by priority so the most pressing targets come first.
- Good goals are SMART:

  Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.
  Also stated positively, under the client's control, and ordered by priority.

- Types: ultimate vs intermediate vs immediate (session) goals; outcome vs process goals; short- vs long-term.
- THEORETICAL VIEW Goal Attainment Scaling (Kiresuk & Sherman, 1968): each goal is scaled from -2 (much worse) to +2 (much better than expected) for individualised measurement.
- THEORETICAL VIEW Goal consensus is a component of the therapeutic alliance (Bordin) and predicts outcome. [Gabbard, PDF p.58]
- Revisiting: goals are revised as new information emerges or progress occurs.

MNEMONIC

SMART

Specific, Measurable, Achievable, Realistic/Relevant, Time-bound — the test of a well-formed therapy goal.

WHY STRUCTURE AND GOALS MATTER

- Structure gives direction, safety and efficiency and keeps therapy within its agreed frame.
- Clear goals increase motivation and let both parties recognise when therapy is complete.
- Collaborative goal-setting is itself a relationship-building, alliance-strengthening act.

Good goals in practice

- Stated positively (what the client will do or have, not only what to stop).
- Under the client's own control rather than dependent on others.
- Ordered by priority, so the most pressing or achievable goals come first.
- Broken into ultimate, intermediate and immediate (session) targets.
- Reviewed regularly and revised as new information emerges or progress occurs.

ANSWER SKELETON

- Explain why structure matters (direction, safety, efficiency).
- Describe the beginning, middle and end phases.
- State how goals are set and the SMART criteria.
- Add Goal Attainment Scaling and goal consensus as an alliance factor.

PRACTICE QUESTIONS

- Discuss structuring of therapy and setting of goals.
- Explain SMART goals with reference to psychotherapy.

11 Pre- and post-assessment

A baseline (pre) gives diagnosis, severity, risk and a benchmark; the post-assessment evaluates change; follow-up checks maintenance. Repeated monitoring lets therapy be adjusted (feedback-informed treatment).

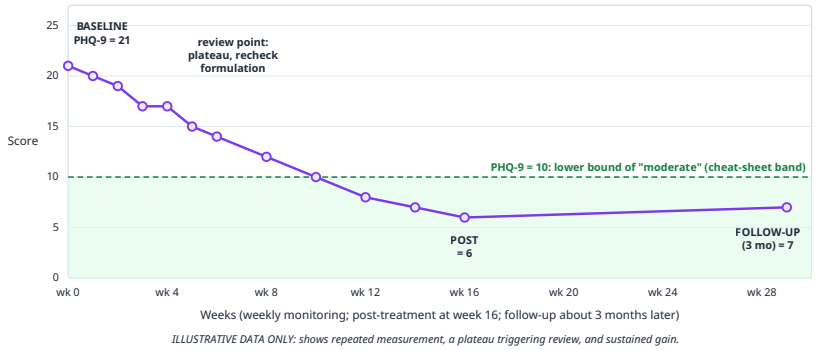


Figure 9.1 Baseline, monitoring, post and follow-up on one measure (hypothetical client; PHQ-9 bands from the repository cheat sheet).
Source/basis: Logic from Beck 2011 PDF p.125, 143, 350, 370; Lister-Ford 2007 PDF p.118; PHQ-9 bands: Rating Scales Cheat Sheet PDF p.78

Why assess before, during and after

- Pre-assessment establishes a baseline (the client's starting level before therapy, used as the benchmark for later comparison), giving diagnosis, severity, risk and strengths against which all change is judged.
- Post-assessment, conducted at the end of therapy, measures how far the client has changed from baseline and whether the agreed goals have actually been met.
- Follow-up assessment some weeks or months later checks whether gains have been maintained or whether the client has begun to relapse and needs further support.
- Ongoing session-by-session monitoring with brief measures, such as the OQ-45, CORE-OM or the Outcome and Session Rating Scales, feeds results back so therapy can be adjusted in real time.
- This continuous feedback, known as feedback-informed treatment (routinely using the client's own outcome scores to guide and adjust therapy), reduces deterioration, especially for clients flagged as not on track.
- Repeated measurement also supports accountability and supervision, because objective scores give a shared, less biased picture of progress than clinical impression alone.

Methods and measures

- Methods: structured/unstructured interviews, behavioural observation, self-report and clinician-rated scales, projective tests when relevant, collateral information, physiological measures, client self-monitoring.
- Commonly used measures:

  Depression: BDI-II, PHQ-9, HAM-D.
  Anxiety: BAI, GAD-7, HAM-A; OCD: Y-BOCS; PTSD: PCL-5 / CAPS-5.
  General distress: SCL-90-R, GHQ; well-being/QoL: WHO-5, WHOQOL-BREF.
  Functioning: GAF, WHODAS 2.0; global impression: CGI; alliance: WAI.

Methods in sentences (exam form)

- Assessment draws on multiple methods, including clinical interviews, behavioural observation (systematically watching and recording what the client actually does), self-report questionnaires and clinician-rated scales.
- Self-report scales (questionnaires the client completes about their own symptoms) such as the PHQ-9 and GAD-7 are quick, standardised and easy to repeat, making them ideal for tracking change over time.
- Clinician-rated scales (measures scored by the therapist from interview and observation) such as the Hamilton scales add an expert perspective that can complement the client's own account.
- Collateral information (accounts from family, teachers or other professionals who know the client) can be sought with consent to give a fuller and more balanced picture of functioning.
- The same measures should be used before and after therapy so that pre and post scores are directly comparable and genuine change can be distinguished from noise.
- Measures must be chosen for their reliability and validity and for their fit with the client's language, culture and literacy, because an unsuitable test yields misleading numbers.

GUIDELINE WHO-5 and WHODAS 2.0 are WHO instruments for well-being and functioning. [WHO]

Two widely used screeners and their severity bands.

Scale	Construct	Severity bands
PHQ-9	Depression (9 items)	0–4 none; 5–9 mild; 10–14 moderate; 15–19 moderately severe; 20–27 severe [cheat sheet, PDF p.78]
GAD-7	Generalised anxiety (7 items)	0–4 minimal; 5–9 mild; 10–14 moderate; 15–21 severe [cheat sheet, PDF p.80]

Evaluating change

- Statistical significance (a result unlikely to be due to chance across a group) answers whether a treatment works on average, but it does not tell you whether an individual client meaningfully improved.
- Clinical significance (a change large enough to matter in the client's real life) asks whether this particular client crossed from the dysfunctional into the functional range on the measure used.
- THEORETICAL VIEW The Reliable Change Index (Jacobson & Truax, 1991) tests whether an individual's change exceeds measurement error, with a value above 1.96 taken as reliable rather than random fluctuation. [Jacobson & Truax 1991]
- Effect size (a standardised number showing how large a change is, such as Cohen's d) and simple percentage improvement give a sense of the magnitude of change beyond mere statistical testing.
- Single-case designs (research designs that track one client repeatedly, such as AB, ABAB or multiple-baseline) let the clinician evaluate change for an individual rather than only for a group.
- Change should be judged on several measures and on the client's own account of functioning, because any one score can move for reasons unrelated to genuine therapeutic gain.

MNEMONIC

BE-MAF

Baseline (pre), Evaluate at end (post), Monitor each session, Appraise change (clinical vs statistical, RCI), Follow-up to check maintenance — the measurement points and tests of change.

INTERPRET WITH CAUTION

- Placebo, regression to the mean, practice effects, response bias.
- Use reliable, valid measures suited to the client's language, culture and literacy.
- Use the same measures before and after; share results with the client.

ANSWER SKELETON

- State the purpose of pre, post and follow-up assessment and ongoing monitoring.
- List methods and commonly used measures (with PHQ-9 and GAD-7 bands).
- Explain statistical vs clinical significance and the Reliable Change Index.
- Note cautions in interpreting change.

PRACTICE QUESTIONS

- Discuss pre- and post-assessment in psychotherapy.
- Explain how change in therapy is evaluated (RCI and clinical significance).

12 Evidence-based practice (EBP) of therapies

EBP integrates the best research with clinical expertise in the context of the patient's characteristics, culture and preferences.

Core ideas in sentences (exam form)

- GUIDELINE The APA (2006) defines evidence-based practice as the integration of the best available research with clinical expertise in the context of the patient's characteristics, culture and preferences. [APA EBPP 2006]
- The idea was borrowed from evidence-based medicine (Sackett's approach of using the best current research to inform clinical decisions) and adapted so that psychology weighs research alongside judgement and client values.
- Empirically supported treatments (specific therapies shown effective for specific disorders in controlled trials) form one pillar, judged by criteria such as those of APA Division 12 (Chambless and Hollon, 1998).
- A treatment is called well-established when at least two good independent trials show it superior to placebo or another treatment, using manuals and clearly defined client samples.
- The hierarchy of evidence (a ranking of study designs by how far they guard against bias) places systematic reviews and randomised trials above cohort studies, single-case designs and expert opinion.
- A key distinction is between efficacy (how well a treatment works under controlled trial conditions) and effectiveness (how well it works in ordinary, messy clinical practice), since trial results do not always transfer.
- EBP is carried out as a repeating cycle, namely asking a clear clinical question, searching for the best evidence, appraising it, integrating it with expertise and client values, applying it and evaluating the outcome.
- GUIDELINE Definition (APA Presidential Task Force, 2006): the integration of the best available research with clinical expertise in the context of patient characteristics, culture and preferences. The concept came from evidence-based medicine (Sackett). [APA EBPP 2006]
- THEORETICAL VIEW Empirically supported treatments (ESTs):

  APA Division 12 (Society of Clinical Psychology) criteria, Chambless & Hollon (1998).
  Well-established: >= two good between-group studies by independent teams (or a large single-case series) showing superiority to placebo or another treatment, with manuals and clear samples.
  Probably efficacious: a lower tier of evidence.
- Hierarchy of evidence: systematic reviews/meta-analyses of RCTs > RCTs > controlled non-randomised > cohort/case-control > single-case > expert opinion.
- Efficacy (works in controlled trials) vs effectiveness (works in real practice) vs clinical utility (feasible, acceptable, cost-effective).
- Examples of well-supported treatments:

  CBT for depression, panic, social anxiety, GAD; ERP for OCD.
  Prolonged exposure, CPT and EMDR for PTSD; behavioural activation and IPT for depression.
  DBT for borderline personality disorder; family-based treatment for adolescent anorexia.
  Parent management training for child conduct problems; CBT-I for insomnia.
  Social skills training, family psychoeducation and cognitive remediation for schizophrenia.
- GUIDELINE Guidelines: NICE (UK), APA, and WHO mhGAP recommend specific treatments. [WHO mhGAP]
- GUIDELINE WHO mhGAP targets non-specialist settings so that evidence-based treatments reach low-resource populations; task-sharing lets trained lay providers deliver brief, manualised care under supervision.
- GUIDELINE India: culturally adapted low-intensity care by lay counsellors — the Healthy Activity Programme (PREMIUM trials, Goa) for depression and the Thinking Healthy Programme for perinatal depression — shows evidence-based care can be scaled up.
- Steps of EBP: ask a clear clinical question » search best evidence » appraise » integrate with expertise and client values » apply » evaluate.

The outcome debate and common factors

- THEORETICAL VIEW Eysenck (1952) provocatively claimed that psychotherapy was no more effective than spontaneous remission (natural recovery that happens over time without any treatment), which stung the field into gathering better outcome evidence.
- THEORETICAL VIEW Smith and Glass (1977) answered with a meta-analysis (a statistical pooling of many separate studies into one overall estimate) of 375 studies, finding a mean effect size of about 0.68 favouring therapy.
- That effect size means the average treated person ended up better off than roughly seventy-five per cent of untreated people, providing strong aggregate evidence that psychotherapy works.

- THEORETICAL VIEW The so-called Dodo bird verdict (Rosenzweig; Luborsky) holds that all bona fide (genuine, properly delivered) therapies produce about equal benefit, which points towards shared common factors rather than unique techniques.
- This debate matters for practice because it warns against assuming that a favoured technique is the main active ingredient when the relationship and the client may matter more.

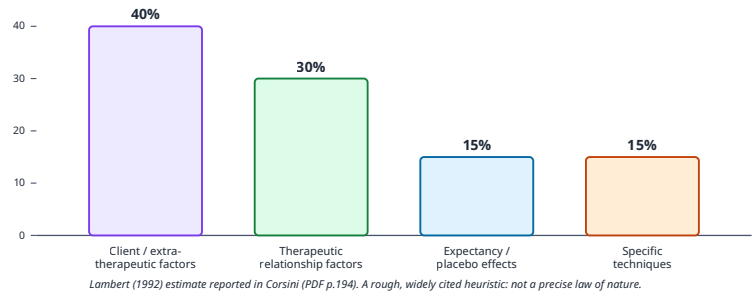


Figure 10.4 Why EBPP must include the relationship and the client, not only technique.
Source/basis: Corsini & Wedding 2011 PDF p.194 (Lambert 1992)

- THEORETICAL VIEW Common factors (Lambert):

  Therapeutic relationship (alliance, empathy); client expectations and hope (placebo).
  A healing setting and rationale; a credible ritual; new learning and mastery.
  Lambert (1992) estimates: extratherapeutic/client 40%, relationship 30%, expectancy 15%, technique 15% (approximate and debated). [Corsini, PDF p.194]
- THEORETICAL VIEW Wampold's contextual model argues these factors, more than specific techniques, explain most benefit.
- Balance: evidence-based techniques work best delivered within a strong relationship.

THEORETICAL VIEW Lister-Ford discusses EBP and routine outcome monitoring as complements to RCTs. [Lister-Ford, PDF p.114-118]

GUIDELINE Corey summarises the APA evidence-based-practice policy and its three pillars (research, expertise, client values). [Corey, PDF p.82]

MNEMONIC

EBP = 3 circles

Best research evidence + clinical expertise + patient characteristics, culture and preferences.

CRITICISMS AND CHALLENGES OF EBP

- RCT samples often unrepresentative (strict exclusion, no comorbidity); limited generalisability to diverse and Indian populations.
- Manuals can reduce flexibility; researcher-allegiance effects; publication bias.
- Under-emphasis of common factors and the therapist's contribution.
- Outcome focus on symptoms rather than well-being or meaning.
- Practice-based evidence, routine outcome monitoring, cultural adaptation and task-shifting are suggested remedies.

ANSWER SKELETON

- Define EBP (APA 2006) and its three pillars.
- Explain ESTs and the hierarchy of evidence; give examples.
- Summarise the outcome debate (Eysenck, Smith & Glass, Dodo bird).
- Explain common vs specific factors (Lambert) and the criticisms of EBP.

PRACTICE QUESTIONS

- Explain evidence-based practice in psychotherapy.
- Discuss the common-factors vs specific-factors debate.
- Critically evaluate the evidence-based-practice movement.

Source note and honest limits

Honesty note: no laws, citations, clause numbers or page numbers in these notes are invented. The following limits apply and are disclosed so you can verify independently.

- SECONDARY SOURCE IACP (Indian Association of Clinical Psychologists): the association's code of conduct could not be read from a primary source (iacp.in was not reachable). IACP is mentioned only in general terms (a members' code exists and broadly aligns with APA/RCI duties). No IACP clause numbers are quoted.
- Page citations use the PDF page index inside each repository file, not the printed page number; this convention is stated so every citation is reproducible.
- Primary texts read in full and relied on here: RCI Act 1992, Mental Healthcare Act 2017, POCSO Act 2012, ICMR 2017 ethical guidelines (research), and the APA Ethics Code 2017, Record Keeping Guidelines and Telepsychology Guidelines.
- Several repository files (for example scanned test manuals) are image-only PDFs with no text layer; they were not OCR'd and are not cited.
- Statements about Indian law are for study; in practice read the current text and seek advice, as laws change.

Key sources & references

Key sources (repository files and primary legal/ethical texts).

- Wolberg, L. R. — The Technique of Psychotherapy (definitions; [Wolberg, PDF p.29]).
- Corey, G. — Theory and Practice of Counseling and Psychotherapy (ethics [Corey, PDF p.74-77]; EBPP [Corey, PDF p.82]).
- Beck, J. S. — Cognitive Behavior Therapy: Basics and Beyond (session structure [Beck, PDF p.82 & 122-123]).
- Jongsma, A. E. et al. — The Complete Adult Psychotherapy Treatment Planner (6-step planning [Jongsma, PDF p.24-26]).
- Gabbard, G. O. — Long-Term Psychodynamic Psychotherapy (alliance [Gabbard, PDF p.58]; supervision [Gabbard, PDF p. 192-193]).
- Lister-Ford, C. — A Short Introduction to Psychotherapy (EBP [Lister-Ford, PDF p.114-118]).
- Corsini & Wedding — Current Psychotherapies (common factors [Corsini, PDF p.194]; ethics [Corsini, PDF p.594-605]).
- Psychological rating scales cheat sheet (PHQ-9 bands [cheat sheet, PDF p.78]; GAD-7 [cheat sheet, PDF p.80]).
- Primary legal/ethical texts: RCI Act 1992; Mental Healthcare Act 2017; POCSO Act 2012; ICMR 2017; APA Ethics Code 2017; APA Record Keeping & Telepsychology Guidelines; APA EBPP policy (2006).
- SECONDARY SOURCE IACP code of conduct — secondary summaries only; primary text not accessed.

Educational study notes. Not legal or clinical advice. Indian statutes and professional codes change: confirm against the primary text or the regulator before relying on them in practice or examinations.